

Virginia Gray (DOB 23.03.1965)

Date: 28.03.2024 | **Clinic:** Hem/Onc | **MRN:** AML072

Dx: AML, ELN 2022 Intermediate — NPM1-mutated with FLT3-ITD (allelic ratio not high by local assay).

1L regimen: 7+3 induction **started 18.01.2023** → CR; now completing consolidation per institutional protocol. (*Midostaurin not used due to prolonged baseline QTc and polypharmacy; monitored closely.*)

S: 59F here for routine f/u. Energy good; resumed part-time work as librarian. No fevers/chills, gum bleeding, melena/hematochezia. Reports mild fatigue last week of each cycle; mild nausea days 1–3 controlled with ondansetron. Denies neuropathy/visual changes. Vaccines up to date. No new meds.

O:

- **VS:** BP 120/74, HR 76, afeb, SpO₂ 99% RA, wt stable. ECOG 0–1.
- **HEENT:** OP clear, no thrush; conjunctivae pink. **Nodes:** none.
- **CV/Resp:** RRR; lungs clear.
- **Abd:** soft, NT/ND, no HSM.
- **Skin:** no petechiae/ecchymoses.
- **Line:** none (PICC removed after consolidation #1).

Key History Recap:

- **Dx 11.01.2023:** BM 68% blasts; flow CD34⁺/CD117⁺/HLA-DR⁺; NPM1 mut+, FLT3-ITD+. Karyotype otherwise normal.
- **Induction (7+3, 18–24 Jan 2023):** Complications: FN x1 (cultures negative), grade-2 mucositis; no TLS.
- **Day-28 BM (15.02.2023):** CR morphologic; **NPM1 MRD** 4.1×10^{-4} (low-positive).
- **Consolidation:** Intermediate-dose cytarabine (IDAC) ×3 cycles (Mar–Jun 2023). MRD declined: 6/2023 1.2×10^{-5} ; 10/2023 **undetectable**.
- **Maintenance:** None (shared decision owing to persistent MRD negativity and QTc concerns).

Labs today (28.03.2024):

- **CBC:** WBC **4.4** (ANC 1.9), Hgb **12.1 g/dL**, Plt **$226 \times 10^9/L$** , MCV 91.
- **CMP:** Na 138, K 4.1, Cr **0.92**, AST/ALT **21/19**, Tbili 0.5, albumin 4.1.
- **LDH:** 176.
- **ECG:** QTcF **472 ms** (stable vs baseline 470–475).
- **MRD:** **NPM1 qPCR undetectable** (PB, 03.2024). **FLT3-ITD** not detected on post-therapy NGS (PB).

Toxicities/Supportive Care:

- Myelosuppression predictable; no transfusions since Aug 2023.
- Antimicrobial prophylaxis only during nadirs (levofloxacin; acyclovir throughout cycles). Mold-active azoles avoided due to QTc/drug-interaction risk; used **micalfungin** briefly during deepest neutropenia.
- Fertility: post-menopausal; contraception not required.
- Mood/sleep: mild anxiety early in treatment; resolved with CBT-i handouts and sleep hygiene.

PMH/Current meds:

- **Hypertension** — amlodipine **5 mg qd**.
- **Hypothyroidism** — levothyroxine **50 µg qAM** (TSH 2.4).
- **Hyperlipidemia** — pravastatin **20 mg qHS** (chosen for minimal interactions).
- **GERD** — famotidine **20 mg qHS PRN**.
- **PRN:** ondansetron 8 mg q8h, senna at bedtime, acetaminophen.

A: AML **NPM1-mut/FLT3-ITD+**, in remission with **serial MRD negativity** since Oct 2023. Completing planned first-line program without midostaurin due to QTc risk; counts and end-organ function stable. No clinical or laboratory evidence of relapse.

P:

1. **Surveillance pathway** now that consolidation is complete: clinic + CBC q3 months in year 2; **NPM1 qPCR** q3–6 months initially, then spacing if negative; **re-stage marrow** only if cytopenias, abnormal smear, or MRD rise.
2. **Cardiac/QTc:** Continue to avoid QT-prolonging meds where possible; maintain $Mg^{2+} > 2.0$ and $K^+ > 4.0$; repeat ECG q3–4 months or with new meds.
3. **Infection precautions:** Prompt evaluation for fever ≥ 38.0 °C; vaccinations per schedule (influenza annually; COVID booster; pneumococcal as indicated).
4. **Lifestyle:** Blood pressure and lipids optimized; encourage 150 min/wk moderate exercise; dental cleanings permitted with standard precautions.
5. **When to call:** Fever, new bruising/petechiae, night sweats, 2-week persistent fatigue, or any syncope/palpitations given QTc baseline.

RTC: 3 months with CBC, CMP, ECG, and PB NPM1 qPCR.

Signature: [Name], NP (supervising MD: [Name])